National Cancer Control Programme (NCCP) 00736 · clinician draft

Nab-paclitaxel · Days 1, 8 and 15 of 28

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nab-paclitaxel · Days 1, 8 and 15 of 28 · each treatment day

Active medicines: Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Routine allergy premedicines are usually not required for nab-paclitaxel. Prior reactions require individual review; treatment is still given where a serious reaction can be managed.
- This is a 100 mg/m² weekly schedule with a planned break. It is not a divided version of the 260 mg/m² three-weekly dose. Its dose-reduction levels are different.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk and the exact source; it is not automatically prescribed by this template.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00736 v2 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required